Nutrition Counseling & Education

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Agenda

- Basics of Education
- Methods of Communication
- Cultural Competence/Communication
- Nutrition Education
  - Theories of Learned Behavior
- Nutrition Counseling
  - Motivational Interviewing and Cognitive Behavioral Therapy
- Formative vs. Summative Evaluations

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Basics of Education

Pedagogy

Education method in which the learner is dependent on the teacher for guidance, evaluation, and acquisition of knowledge. *Ex: RD provides children with examples of how to eat healthy.*

Andragogy

The methods and approaches used in adult education and is directed towards:

1. Need to know info
2. Self-directed learning
3. Expanding experiences and Readiness to learn
4. Problem-centered
5. Internal motivations

self-actualization, gaining experience, and problem-solving.

Ex: RD assists adult client in identifying healthy eating strategy that works for him.

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Teaching Different Populations

Middle and Upper Income	GOAL oriented; if they can't relate = waste of time
Lower Income	Interested in costs
Very poor	Short-term planners
Adolescents	Consider peer pressure, relate to their interests
Elderly	Decreased attention span; short is best

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Methods of Communication

Written	Oral
• More formal • Ex: Emails, texting, pamphlets, posters, handouts • Low literacy participants: 6th grade level • High literacy/general participants: 8th grade level • How to test readability of material: SMOG test	• More personal • Encourages your participants to PARTICIPATE! • Barrier: listening... • Ex: talking on TV, in person presentation, podcast, radio show

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Methods of Communication

Verbal	Non-Verbal
• Personal • Positive rather than negative • Be cooperative instead of authoritarian • Be empathetic instead of neutral • How to test readability of material: SMOG test	• Clothing, dress, appearance • Demeanor • Eye contact, body position • Practitioner should maintain professional non-verbal communication and interpret the client as well

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Effective Communication

Remember!

Effective and competent communication requires:

- The message be understood
- Achieves intended effect
- Ethical and truthful

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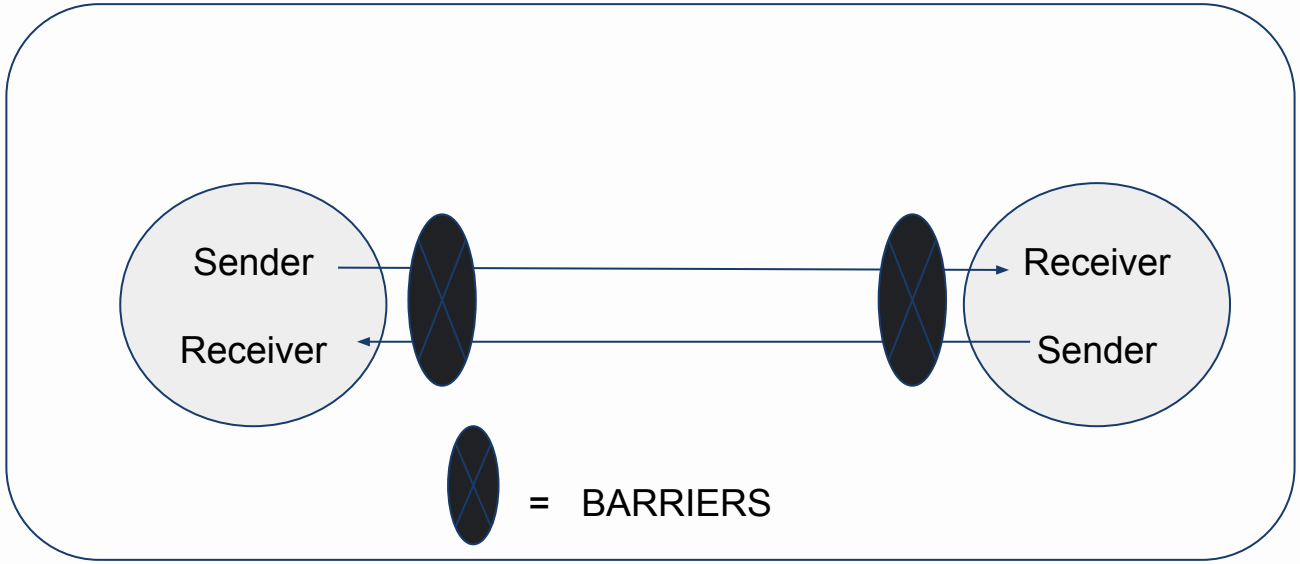
Methods of Communication



Barriers include:

- Room
- Interruptions
- Anxiety, fear, apprehension
- Misunderstanding
- Cultural differences
- Language barrier

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Cultural Communication

The way you communicate depends on who you are talking to / what culture they are from. For example, here is what different types of **kinesics** (physical communication) mean in some cultures:

Good posture and averting eyes =	Sign of respect
Lowering eyes =	Preoccupation, worry
Arms folded on desk =	Dislike/avoidance
Clenched fist =	Anger
Crossing/uncrossing legs =	Anxiety
Moving chair away =	Discomfort

You even have to consider **proxemics**/spatial relationships (amount of space between people when talking), the rate/pitch/volume of your voice (**paralinguistic**), eye contact, and the way you greet people.

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Cultural Communication

Spatial Relationships

Intimate Zone	Less than 18 inches
Personal Zone	18 inches to 4 feet
Social Zone	4 feet to 12 feet
Public Zone	Greater than 12 feet

The appropriate zone will vary depending on the culture/individual!

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Cultural Competence

Definition: “the ability to understand, appreciate and interact with people from cultures or belief systems different from one's own” (APA)

Once we develop cultural competence, we know how to properly address individuals in different cultures, which helps increase satisfaction and adherence to recommendations

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ETHNIC Model

Explain	What do you know about your disease? What are others in your culture doing about this disease?
Treatment	What type of treatments will you be using?
Healers	What types of non-traditional healing are you using?
Negotiation	Work together to find acceptable treatment choices.
Intervention	What feedback do you have on this intervention?
Collaboration	Working with other health care professionals to achieve outcomes.

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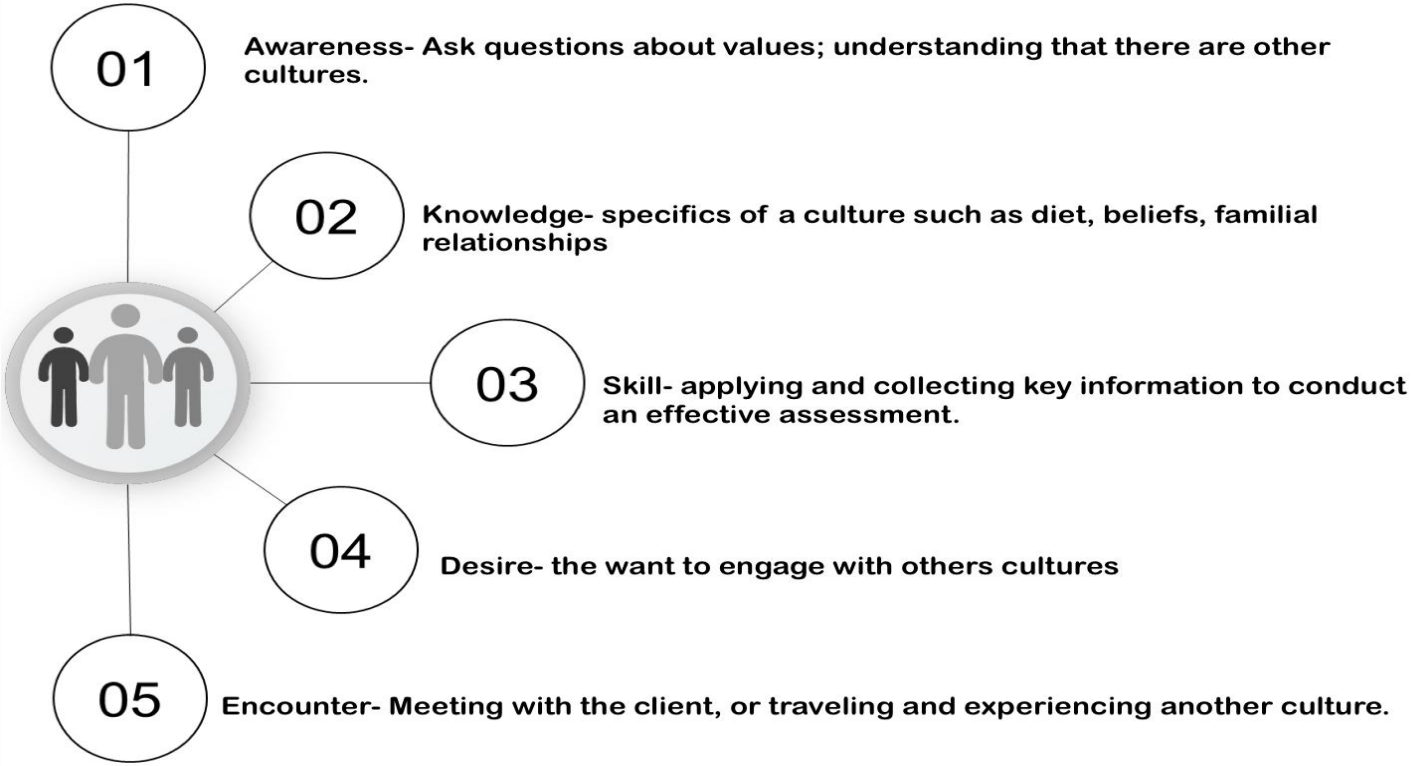
Campinha Bacote Model



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Nutrition Education

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Goals vs. Objectives

Goals

- An achievable outcome that is broad and long-term

Example:

Decrease rates of constipation in the elderly at Johnson ALF

Objectives

- Defines measurable actions in the short term to achieve the overall goal
- S.M.A.R.T. goal:
  - Specific, measurable, achievable, relevant, and time-bound

Example:

At the end of session 1, participants will be able to list 5 foods that are high in insoluble fiber with >80% accuracy

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Practice Question

The SMOG formula determines

A. The readability level of written materials
B. The cultural competence of written materials
C. A and B
D. None of the above

A

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Theories of learned behavior/education

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Educational Theory: Transtheoretical Stages of Change Model

PCPAM

Stage	Description	Intervention
Precontemplation	Unaware or not interested in making a change	Consciousness raising Assess knowledge Increase self-awareness Risks vs. Benefits
Contemplation	Aware they need to make changes NO COMMITMENT TO CHANGE	Assess knowledge Assess thoughts/feelings Increase pros Decrease cons
Preparation	Intends to take action in the next 30 days	Self-efficacy Decision-making Plan goals
Action	Implementing the plan of action for up to 6 months	Control stimulus Reinforcement Self-management
Maintenance	Maintaining healthy habit for 6 months or more	Coping responses Relapse prevention

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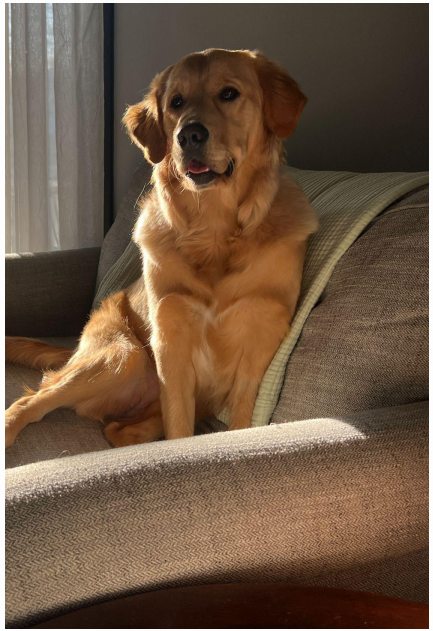
Educational Theory: Operant vs. Classical Conditioning

Operant Conditioning	• The idea that learning is a function of change in overt behavior, and changes in behavior are the result of an individual's response to events in the environment • If stimulus is negative = behavior decreases • If stimulus is positive = behavior increases
Classical Conditioning	• Two stimuli are paired together for a number of trials until a response is elicited • "My morning alarm signals that I need to go workout"



"Classical music" → think of instruments → think of a bell

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Educational Theory: Health Belief Model

Trick to remember all:
2 perceived **S**, 2 perceived **B**, self-efficacy and cues

Perceived Susceptibility

"Since I am Native American, I am more likely to have Diabetes"

Perceived Severity

"Diabetes can put me at greater risk for heart disease"

Perceived Benefit

"If I control my diabetes, I will feel better"

Perceived Barriers

"I don't have enough family support to take care of my diabetes"

Self-Efficacy

"I can successfully check my blood sugar levels after I eat"

Cues to Action

"I can set an alarm on my phone to remind me to exercise"

Behavior change

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Practice Question

Harry tells the RDN: “I know I have a heart issue and should eat healthier, but I don’t really intend to do much about it anytime soon.”

A. Pre-contemplation
B. Contemplation
C. Preparation
D. Action

B

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Practice Question

After attending a nutrition education session on nutrition for osteoporosis, a 70-year-old female client decides she wants to start eating more calcium-rich foods because she has identified herself as being at risk. What component of the Health Belief Model has triggered her behavior change?

A. Perceived benefit
B. Perceived barrier
C. Perceived susceptibility
D. Self-efficacy

C

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Practice Question

Carlos is preparing steaks for his family. Last time he grilled out, he ended up sick with food poisoning from not checking the temperature of the steak before serving. Carlos chooses to check the temperature because of this experience. This is known as:

A. Classical conditioning
B. Operant conditioning
C. Perceived benefit
D. Modeling

B

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Nutrition Counseling

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Counseling Steps

1. Opening

Introduce yourself

Establish Rapport:
- Weather
- Sports
- Local events

Discuss purpose

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Counseling Steps

2. Exploration Assessment

- Open-ended, broad questions most of the time!
- Limit closed questions
- Know the difference between primary (introduce new topics) and secondary (follow up) questions
- No leading questions/questions that reveal your bias (are you eating your 5 portions of f/v per day?)
- BEST types of questions: "Tell me about..." Example: Don't ask "do you exercise?" but ask "tell me about your relationship with exercise..."

- NO "why" questions - threatening/attacking
- LISTEN throughout the interview; note nonverbal cues

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Listening Responses

Clarification	Asking a question after an ambiguous message “What do you mean by...?” “Did I hear you correctly when you said...”
Reflective Listening	Paraphrasing what was just said; focused on feelings of other person “So you feel...” “It sounds like you...” “You're wondering if...” “For you it's like...”
Legitimation	Acknowledges that what they feel is legitimate “You have a right to feel upset, anybody would!”
Feedback	“Testing” your client - lets you know what they know and that they understand what you are telling them “Tell me again, what are 2 foods that are high in fiber?”
Evaluative Response	When you offer advice but don’t solve the problem - seldom helpful “If you eat too much ice cream, you should stop buying it” “if you eat poorly out, stop going to restaurants”
Hostile	Antagnozies/humiliates the client “Maybe you are not losing weight because you are not trying hard enough”
Reassuring	Does not work!! “Everything will be okay” “Don’t worry about that”
Probing	Encourages conversation, makes clients think. “Can you tell me more about that?” “Why do you think that?” “What would happen if...”
Understanding	Recreating person’s message in your frame of reference; putting yourself in their shoes “You seem to be saying you are feeling...”
Affirmation	“Good job!” “Great, that shows you are dedicated!”
Self-Disclosure	“This happened to me, too!”

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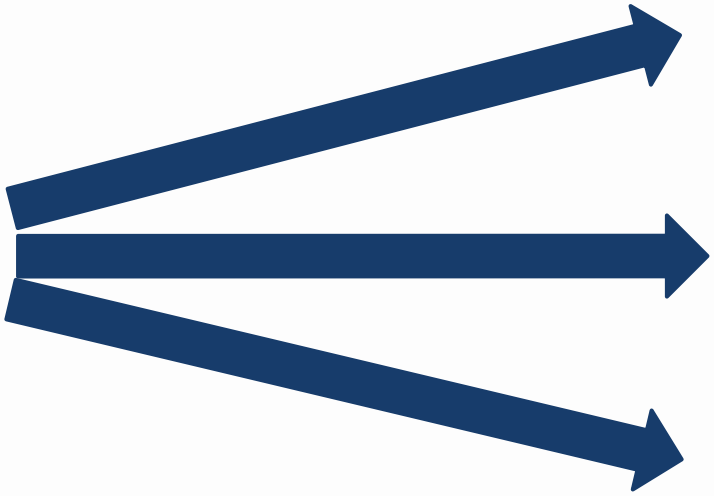
Counseling Steps

3. Closing



- Express appreciation
- Summarize points and goals
- Declare completion

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Practice Question

An RD provides medical nutrition therapy to a Latin client on dialysis with hyperkalemia, Stella. When Stella discloses she's angry because a low-potassium diet limits her ability to eat copious amounts of her favorite cultural fruits, the RD responds, "What I hear you saying is that limiting the amount of guava, bananas, and mangos you can eat daily makes you mad." This is an example of what type of listening response?

A. Legitimation
B. Probing
C. Reflective Listening
D. Affirmation

C

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Practice Question

A patient reveals to the RDN that he is having trouble eating whole grains because he dislikes the taste. The RDN responds, "I also had this problem when I started eating whole grain pasta!" This is an example of what listening response?

A. Legitimation
B. Reassuring
C. Self-disclosure
D. Affirmation

C

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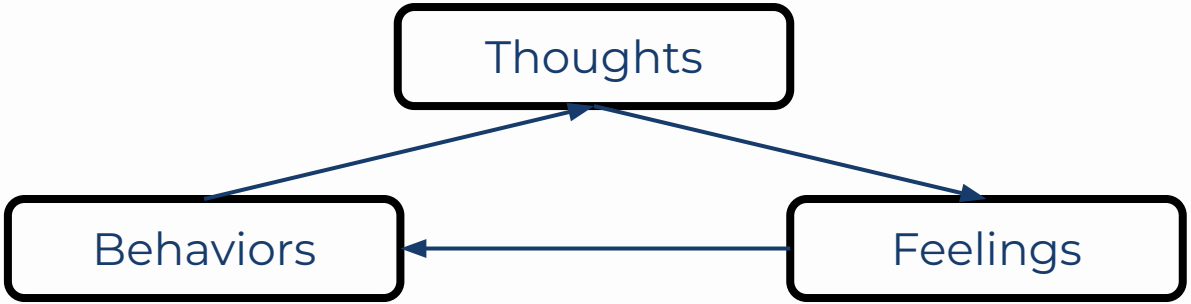
Cognitive Behavioral Therapy (CBT)



- Removing negative self-talk and eliciting change talk
- Identifying behaviors and thoughts that are negative and applying strategies to change those
- From “I can’t do it” to “I want to control my health to feel better!”
- NO ROOM for negativity!!!

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Motivational Interviewing (MI)

- Goal-oriented communication style
- Idea is to increase motivation and readiness change in clients/patients, *allowing them to evoke a desire to change on their own*
- Client-centered
- Does NOT give advice to the client - *decisions are left up to them!*

The focus is *CLIENT AUTONOMY*

→ Improves change initiative

→ Maximizes strengths

→ Reduces resistance and ambivalence

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Motivational Interviewing (MI)

"**A**MANDA **S**INGS **R**OCK **E**VERY **D**AY"

AVOID ARGUMENTS: Avoid Confrontation

SUPPORT SELF-EFFICACY: Reinforce hope and optimism in the patient; promote belief in their ability to do the skill needed

ROLL WITH RESISTANCE: Acknowledge reluctance to change "You are not happy to be here today"

EXPRESS EMPATHY: Being compassionate, accepting

DEVELOP DISCREPANCY: Help clients see difference between their values and their behaviors "you want to balance your blood sugar, but you need to drink juice with every meal"

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Motivational Interviewing (MI)

STEPS: EFEP

Engage	• "Hi" phase. Establish rapport/comfort/safety/empathy
Focus	• "What" phase. Finding a clear direction/goal when it's not clear from the beginning. What is the particular goal for change in this client? What is the target behavior?
Evoke	• "Why" phase. Evoke a person's own motivations to change, and reinforce it to help build their motivation for change around the focused target behavior. • Elicit ***"change talk"*** = any speech that favors movement toward change. Try to keep them away from "***sustain talk"*** (talking about why they can't change)
Plan	• "How" phase. Developing a commitment to change and formulating a specific plan of action

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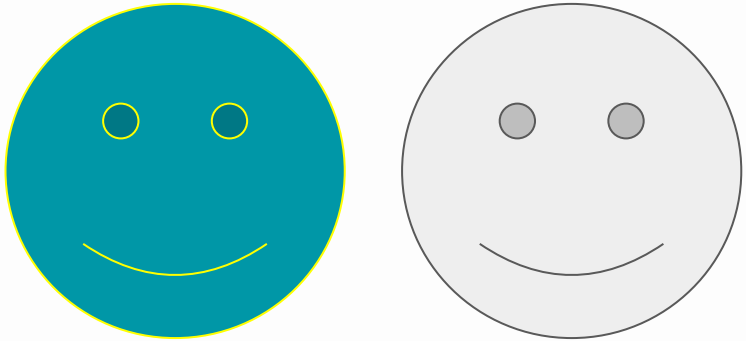
Motivational Interviewing (MI)

What is something simple you could see yourself doing to start optimizing your hydration?

-RD



I guess I could start bringing a water bottle with me to work...

-Client

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Social Cognitive Theory

Modeling: Observing behaviors and behaving based on what you see others doing. Helps determine human behaviors.

Cognitive Factors
- Knowledge
- Expectation
- Attitudes

Environment Factors
- Social Norm
- Community
- Influence on others

Behavior Factors
- Skill
- Practice
- Self-Efficacy

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Steps to Effective Education

1. Assessing learning needs (pre assessment)
2. Writing feasible and measurable performance objectives
3. Determining educational content and Learning Domain
   a. Cognitive (Knowledge)
   b. Affective (Attitudes)
   c. Psychomotor (Skills)
4. Select methods, techniques, materials and resources for the individual or group
5. Implementing the intervention
6. Evaluate progress and outcomes
7. Document results and outcomes of the education

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Evaluating your Education

Formative Evaluation

- Done BEFORE or DURING the course of education
- Identifies learner misconceptions, struggles, and learning gaps DURING the educational process

Examples:

Focus Group

Pilot Test

Pre-Test

Quizzes

Summative Evaluation

- Done at the END of your teaching
- To see if you accomplished your goals/objectives
- Did you achieve what you set out to do?

Examples:

Post-Test

End of semester exam

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Practice Question

During a nutrition counseling session with a client, the client states: “As much as I would like to bring a healthy lunch with me to work, I can’t because I don’t have time to cook.” This is an example of:

A. Self-efficacy
B. Change talk
C. Sustain talk
D. Cues to action

C

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Practice Question

A dietitian is implementing a 6 session program at the local YMCA for older adults about nutrition for osteoporosis. Which of the following is NOT considered a formative evaluation?

A. A take home quiz provided to participants after session 3
B. A quiz provided to participants after session 6
C. A quiz provided to participants before session 1
D. Asking participants in the middle of the class to repeat what the dietitian just taught

B

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Practice Question

A client explains to the dietitian “I know my blood sugar is high, but this is not the right moment to make changes in my life.” The RDN responds by asking, “What are the advantages and disadvantages of making changes now?” What principle of motivational interviewing is this?

A. Develop discrepancy
B. Roll with resistance
C. Avoid arguments
D. Support self-efficacy

A

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